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Submissions

Page Charge

A \$70 per-page charge will be assessed for publication in *Arthritis & Rheumatology*. Instructions for payment will be sent to authors along with the proofs.

- There is no page charge for solicited articles, letters to the editor, or clinical images.
- There is no page charge if the first author or corresponding author has been a member of ACR (American College of Rheumatology) / ARP for at least three years.
- In extreme cases of inability to pay, the page charge may be reduced or waived; this should be requested at the time the article is accepted.

Fee for publication of color figures. Articles may include one page of color in the print issue at no charge to the author. For subsequent pages of color in the print issue, the author will be billed \$500 per page. Authors may opt to have some of their color figures appear in black and white in the print issue, and in color in the online issue. There is no charge to authors for publication of color figures on multiple pages within an article in the online issue only.

How to Submit

following:

- Type of manuscript (e.g., full-length article, brief report, or letter to the editor);
 - manuscripts should not be classified as editorials or special articles unless they have been specifically invited.
- Corresponding author's address, telephone number, fax number, and e-mail address.
- Confirmation that the manuscript:
 - Has not been submitted or is not simultaneously being submitted elsewhere,
 - Is not at the time of submission under consideration by another journal or other publication,
 - Does not contain data that has been or will be published elsewhere while the manuscript is under review by the journal, unless rejected by *Arthritis & Rheumatology* or withdrawn by the author(s).

Eligibility requirements:

- If a manuscript is in "revision suggested" status with another journal, it is interpreted to be still under consideration by that journal; for the manuscript to be submitted to *Arthritis & Rheumatology*, it must first be officially withdrawn from consideration with the other journal.
- A manuscript that is in "revision suggested" status with *Arthritis & Rheumatology* is deemed to be under consideration and may not be submitted to another journal unless the submission is withdrawn from *Arthritis & Rheumatology*.
- Publication in proceedings or transactions of meetings or symposium volumes is considered to be prior publication that renders the manuscript ineligible.
- Publication on a preprint server is not considered to be prior publication, and the journal accepts the submissions of manuscripts that have been previously posted on a preprint server.
- The publication of data in abstracts, and presentation in oral or poster sessions at meetings, do not constitute previous publication.
- Each individual named as an author must meet the journal's criteria for authorship (listed in the Authorship Criteria section below).

Supplementary Material

Supplementary material will be considered for publication in the online edition of *Arthritis & Rheumatology*. This is **NOT** copyedited by *Arthritis & Rheumatology* staff and should be carefully reviewed by the authors before the final version of the manuscript is submitted.

- Supplementary material should consist of tables, figures, or short videos.
- Supplementary text describing methodologic information that is too lengthy for inclusion in the body of the published manuscript also may be considered.
- Each supplementary item should be mentioned in the manuscript (in chronological order if there are numbered supplementary tables or figures).

If there is supplementary material that does not meet the above criteria but that the author believes is critical to the understanding of the article's content, a request for a waiver of the criteria may be made during the review/revision process and will be considered by the Editor.

For submitted supplementary material that is not accepted for publication in the online edition of the journal, the author may state in the manuscript that the information is available from the corresponding author upon request. However, the material may not be referred to using the term "supplementary."

Authors may suggest potential reviewers for manuscript or individuals authors would prefer not to review. (Editors will consider these suggestions but are not bound by them.)

Arthritis & Rheumatology requests that peer reviewers adhere to a set of basic principles during the peer-review process in research publication.

You should not have a financial, personal, or intellectual conflict of interest that may diminish your ability to provide a fair review.

Please refer to the [Ethical Guidelines for Peer Reviewers](#) prepared by the Committee on Publication Ethics, for further information on how to be objective and constructive in the peer-review process.

Author corrections on proofs

Arthritis & Rheumatology will provide corresponding authors with page proofs and request review for errors and editorial queries.

Publication of the article may be delayed if the proofs are not sent back in a timely manner.

Manuscript Formatting

Formatting Summary by Article Type

<i>Article Type</i>	<i>Word Limit; Reference Limit; Figure/Table Limit</i>	<i>Abstract Required</i>
Full Length	4200; 50; 6	Yes, structured, 250 words
Brief Report	2500; 15; 3	Yes, structured, 250 words
Review	5000; 85; 6	Yes, unstructured, 250 words; Case reports: narrative review, 250 words
Expert Perspectives on Clinical Challenges	5000; 85; 6	Yes, unstructured, 250 words

Notes from the Field	1500; 15; 1	No
Editorial	1500; 15; 1	No
Letter to the Editor	500; 10; 2	No
Letter to the Editor	500; 10; 1	No
Reply	500; 5; 1	No
Clinical Image	300; 3; 2	No

Format and organization

- Type all pages of the manuscript, including those containing references, tables, and figure legends, double space in 12-point type, with 1-1½-inch margins.
- Number all sheets in succession, including references, tables, and figure legends. Title page is page 1.
- On the first page, type the running head (short title for top of each page), title (which cannot include any acronyms), names of the authors and their academic degrees, grants or other financial supporters of the study, address for correspondence and reprint requests, and corresponding author's telephone and fax numbers and e-mail address.
- Also on the first page, again indicate any financial support or other benefits from commercial sources for the work reported on in the manuscript, or any other financial interests that any of the authors may have, which could create a potential conflict of interest or the appearance of a conflict of interest with regard to the work.

Manuscripts submitted to *Arthritis & Rheumatology* receive a preliminary quality review by the editorial staff and may be returned if length or formatting requirements are not met. All manuscripts are initially reviewed by members of the editorial board and a substantial proportion of manuscripts are returned without external review if judged to be not competitive or in line with the journal's content range.

Only a minority of submitted manuscripts achieve sufficient priority for acceptance, and editorial decisions balance assessments of research quality with considerations of novelty, translational impact (in the case of basic science studies), and interest for the journal's broad readership. Given the competition for publication space, rebuttals to editorial decisions are not considered except in cases of a clear scientific error in the review.

text changes made in the paper, as opposed to just providing page locations where changes were made.

References

- Compile references numerically according to the order of the citation. Consult recent issues of *Arthritis & Rheumatology* for reference style. Reference numbers cited in the text should be superscripted and called out in numerical order.
- For references to journal articles, include the first 3 authors, followed by “et al” if there are more than 3 authors. The author names are followed by the title (with only the first word capitalized), the abbreviated name of the journal (use abbreviations that conform to those in the [National Library of Medicine journals database](#)), the year, volume number (without issue number), and page range.
- Example: Kennedy A, Ng CT, Biniecka M, Saber T, Taylor C, O’Sullivan J, et al. Angiogenesis and blood vessel stability in inflammatory arthritis. *Arthritis Rheum* 2010;62:711-21. If the journal material being cited is an editorial, review, abstract, or letter, indicate this in brackets at the end of the title.
- For chapters of books that have an editor(s) and individually authored chapters, include the names of the chapter authors, followed by the chapter title, the book editors, the book title, the city of publication, publisher name, year of publication, and chapter page range. Example: Katz JN. Developments in surgery for rheumatic and musculoskeletal disorders. In: Pisetsky DS, editor. *The ACR at 75: a diamond jubilee*. Hoboken (NJ): Wiley-Blackwell; 2009. p. 87-91.
- For references to electronic publications, include the author or organization as applicable, title as applicable, and the URL; prior to submission, please confirm that the URL is active.

All criteria and guidelines mentioned in the article must be listed in the references.

Article Types

For detailed instructions on formatting manuscripts by article type, [review this document](#).

Illustrations

Initial submission

At **initial submission**, figures may be uploaded as part of the manuscript Word file, compiled into a single .pdf, or uploaded as individual figure files. As long as figures are legible and do not exceed the maximum number allowed for a given manuscript type, no further formatting will be required.

Revision

For revision submissions, the following figure formatting requirements will be necessary:

1. *File format*. Images should be uploaded as separate files following the manuscript Word file, in one of the following formats: .tiff (preferred), .eps, or high-resolution .pdf. The image file should not include the figure title or figure legend. Figure legends (including the figure title as the first line of the legend) should appear only at the end of the manuscript Word file. A figure that contains more than one panel should be uploaded as a single combined file that contains all of the panels on 1 page, i.e., Figures 1A, 1B, and 1C must be submitted already combined on 1 page, not 3 separate pages. Panels should be arranged to minimize blank page space.
2. *Resolution*. For line drawings, charts, and other figures, the minimum acceptable resolution is 300 dpi. Authors are strongly encouraged to submit gels, blots, radiologic images, and microscopy images at an originally acquired resolution of at least 600 dpi. **Note that initially creating a figure**

3. **Size.** A figure, including the legend below it, cannot exceed the page size of the journal (and preferably will occupy substantially less than a full page), and **figures will be reduced to achieve this.** Therefore, **all elements of a figure (labels and other text, symbols, bars and other elements of graphs, scale bars, arrows, asterisks, and key clinical, radiographic, or histologic findings being depicted) must be large enough to remain legible on the printed page after this reduction. The lengthier the legend, the less space will be available for the figure itself; legends should be as succinct as possible and may not exceed 200 words.** Lengthy descriptions of methods should appear in the Methods section of the article and not in figure legends. If a figure and/or its legend is too large for all of the figure elements to remain legible after sizing to fit the printed page, the Editors may require that the figure or some of its elements be deleted or submitted as online-only supplementary material.
4. **Labels, fonts, and text.** Although the letter of a multiple-part figure may be printed directly on the figure, the number should not be (i.e., for Figure 2A, an "A" may appear on the figure, but "2" may not; the number will appear in the text of the figure legend). Do not include parentheses or periods with the labels "A," "B," etc. For figures with multiple panels, please keep the style (e.g., capitalization and other style features) and font consistent across all panels. Use only **Arial or Helvetica** font for all labels, numbers, and any other text on figures. As noted above, use font sizes throughout that will remain legible on the printed page after reduction. Do not use boldface or italic labeling. As a US-based journal, we edit non-US spelling in the text to the US version (e.g., "litre" to "liter," "tumour" to "tumor"). Therefore, for consistency within your published article, please use US spelling on the figures.

Authors submitting composite images must supply high-resolution files for each component of the image.

Ethical requirements for images

1. Images should be minimally processed, faithfully represent the original data, and conform to accepted scientific standards.
2. Images must not be manipulated; no specific feature within an image may be enhanced, obscured, moved, removed, or introduced. The use of software touch-up tools that potentially obscure manipulations is not permitted. Processing to adjust brightness, contrast, or color tone is allowable only when it is applied equally across the entire image and to corresponding controls. Contrast should not be adjusted so that data information disappears or is obscured. In general, image acquisition tools (image analyzers, microscopes) and image processing software should be described in the Figure Legend or the Methods. Key data acquisition parameters (equipment settings, magnifications, etc.) should be included.

Specific figure types

1. **Charts and graphs.** Charts and graphs should include dots or other symbols that show all individual observations on plots whenever possible, but especially in analyses including <20 observations or analyses with skewed distributions. If individual observations are not represented in the plot, then the n values should be included along the x-axis. **Dynamite plunger plots (bar charts with lines representing the SD or SE or IQR, but without individual data points; see example below) or other charts showing only the mean or median and SD or SE or IQR will generally no longer be accepted.**



Dot plots with means and SD or SEM indicated, or box-and-whisker plots (showing, e.g., interquartile ranges, means or medians, other ranges, and outliers) are more appropriate for meaningful representation of the data. Charts and graphs in articles reporting on clinical trials should include a number at each data point in a graph or above each bar in a chart, for ease in determining the exact value being depicted. For pie charts, Venn diagrams, or any other charts or graphs in which different variables are represented by different colors (or different shading such as hatched lines, cross-hatched lines, etc.), ensure that all colors or shadings used are clearly distinguishable from one another (whether in full color or gray scale, and after size reduction). Do not use, for example, a light gray to represent one group and an only slightly darker gray to represent another.

2. **Blots.** For electrophoretic and blotting images, positive and negative controls, as well as molecular size markers, should be included. The antibodies employed must be cited in the Methods and include the manufacturer and product number and be available to the research community. Loading controls, when analyzed, must be from the same electrophoretic analysis. A loading control should be submitted for each blot showing specific changes in experimental gene or protein levels. The display of cropped gels and blots is permitted only if it improves the clarity and conciseness of the presentation. For quantitative comparisons, the figure legend must state that the samples derive from the same experiment and that gels/blots were processed in parallel. Cropped images that juxtapose lanes must have a clear separation or a line that delineates the boundary between the gels or blots. Loading controls (e.g., GAPDH, actin) must be run on the same blot. **For any revised submission that includes cropped gels or immunoblots, upload a separate PDF, PPT, or PPTX file (distinct from any other supplementary material provided) that displays the entire unedited gel. Clearly indicate which bands were used in the figures. These will be archived with the manuscript.**
3. **Clinical, radiographic, or histologic images.** Please avoid including a wide area beyond the area of interest. Images may not be manipulated; no specific feature within an image may be enhanced, obscured, moved, removed, or introduced. **Obtain a release from the patient or legal guardian for any image that includes identifiable features of the patient.** Blinders on the eyes are not a substitute for patient's permission. Authors who do not have a patient release form from their institution or practice may use the form available [here](#).

Other general information

Graphic design assistance for schematic drawings. For articles that contain, or should contain, schematic drawings depicting molecular mechanisms or disease processes, *Arthritis & Rheumatology* offers the services of a graphic designer. The designer will assist authors with creating these drawings or improving an existing drawing. Details can be discussed during the review process. There is no fee for this service.

Fee for publication of color figures. Articles may include one page of color in the print issue at no charge to the author. For subsequent pages of color in the print issue, the author will be billed \$500 per page. Authors may opt to have some of their color figures appear in black and white in the print issue, and in color in the online issue. There is no charge to authors for publication of color figures on multiple pages within an article in the online issue only.

- Include horizontal borders below the title and headings and above the table footnotes only; there should be no horizontal borders separating the individual lines of data in the table body.
- Limit the width of each table (number of columns) such that it will fit in portrait (not landscape) orientation on a journal column (inches) or page (7 inches) and will not exceed the height of the page.
- Tables with sections (e.g., Table 1a, Table 1b) are not acceptable and will be handled as two separate tables unless the information can be logically combined into one table with one set of headings.
- Provide each table with an explanatory title so that it is intelligible without specific reference to the text.
- Provide each table column with an appropriate heading. Indicate clearly any units of measure on a table.
- Lengthy descriptions of methods should appear in the Methods section of the article and not in table footnotes.

Content, acronyms, and terminology

- Do not use new technical words, laboratory slang, words not defined in dictionaries, or abbreviations or terminology not consistent with internationally accepted guidelines. Define any abbreviations the first time they are used.
- A [list of acronyms for many common rheumatology terms](#) has been developed by an international group of rheumatology journal editors.
- Names of acronyms for genes and proteins should comply with their official entries in [Entrez Gene](#) and [UniProt](#), respectively.

Methods section should include, whenever possible, a short paragraph detailing the proportion of patients who satisfy the ACR classification criteria for the particular disease described.

Statistical Reporting

The journal's statistical consultants recommend that statistical best practices be followed in the design and reporting of research studies.

In addition, it is strongly recommended that authors include the appropriate reporting checklist based on the given study type (see [reporting guidelines](#)). This is for use in the review process only, and therefore should be included as a separate document from the manuscript.

- For all studies, the Methods section of the manuscript should contain descriptions of 1) sample size and power considerations for the study; 2) the methods for primary and secondary analyses; and 3) how missing data have been handled (complete case analyses that include only subjects with complete data are rarely appropriate).
- The Results section should include 1) confidence intervals for estimate effect sizes, measures of association, or other parameters of interest and 2) 2-sided *P* values in almost all circumstances where hypotheses are being tested, such as a randomize controlled trial. Results should be presented with no more precision than is of scientific value and is meaningful given the available sample size.
- Further details on statistical reporting for specific study types (clinical trials, observational studies, and preclinical and translational studies) are provided in the journal's [Guide to Statistical Reporting for Specific Study Types in ACR Journals](#).

Arthritis & Rheumatology follows the International Committee of Medical Journal Editors Recommendations for the Conduct, Reporting, Editing, and Publication of Scholarly Work in Medical Journals: <http://www.icmje.org/recommendations/>.

Article policies

Research ethics standards

Human Subjects

- Research carried out with human subjects must be in compliance with the [Declaration of Helsinki](#)
- A statement to this effect must appear in the Methods section of the manuscript, including the name of the body that gave approval, as must a statement confirming provision of written informed consent human subjects (or receipt of a waiver from this requirement).

Animal Subjects

- Methods section must include a statement indicating approval by the appropriate institutional review board or comparable formal ethics review committee.

Funding

- Compliance with Open Access regulations required by funding bodies, such as the National Institutes of Health, is required.

Plagiarism and image manipulation

- The journal reserves the right to subject any submitted text or figures to electronic scrutiny to ensure that text has not been plagiarized and images have not been inappropriately manipulated.

Diversity, Equity, and Inclusion considerations

The American College of Rheumatology and the Association of Rheumatology Professionals are opposed to discrimination on the basis of:

- race
- color
- ethnicity
- sex
- gender identity
- gender expression
- social class
- national origin
- pregnancy
- age
- disability
- creed
- religion
- sexual orientation
- veteran status
- political affiliation

and tables in reports of clinical research, so that all people are treated with dignity and respect.

Furthermore, we ask authors, editors, and reviewers to ensure that differences in results based on group status are correctly attributed to biology rather than to non-biological factors, and to follow the guideline on reporting participant characteristics outlined in the Instructions to Authors.

The information below provides examples of practices in the conduct and reporting of clinical research that the journal will take into consideration. This pursues the goals of increasing the accuracy and applicability of the information reported and ensuring that the language is inclusive, descriptive, and not offensive.

Prior to submission of articles involving human subjects, authors should review the table to ensure that all applicable elements have been addressed. Currently the checklist below does not need to be completed/submitted with the manuscript; however, it will be used by reviewers and editors when evaluating articles involving human subjects.

- The Methods section should include an explanation of how participant race and ethnicity were determined. If self-reported, authors should specify whether individuals answered an open-ended question or from a fixed set of categories.
- NIH terminology should be used for racial and ethnicity categories. Note that these guidelines are specific to US studies. Non-US locations may have different racial and ethnic populations and/or use different terminology, meaning that these guidelines may not fit well. However, non-US studies are expected to follow the intent of these recommendations.
- Authors should use American Indian or Alaska Native, Asian, Black or African American, Native Hawaiian or Other Pacific Islander, and White for race and Hispanic or Latino/a or Not Hispanic or Latino/a for ethnicity. Individuals' self-identified countries of origin can also be reported when appropriate. Please see definitions (NOT-OD-15-089) below:
 - **American Indian or Alaska Native:** A person having origins in any of the original peoples of North and South America (including Central America), and who maintains tribal affiliation or community attachment.
 - **Asian:** A person having origins in any of the original peoples of the Far East, Southeast Asia, or the Indian subcontinent including, for example, Cambodia, China, India, Japan, Korea, Malaysia, Pakistan, the Philippine Islands, Thailand, and Vietnam.
 - **Black or African American:** A person having origins in any of the Black racial groups of Africa.
 - **Hispanic or Latino/a:** A person of Cuban, Mexican, Puerto Rican, South or Central American, or other Spanish culture or origin, regardless of race. The term, "Spanish origin," can be used in addition to "Hispanic or Latino/a."
 - **Native Hawaiian or Other Pacific Islander:** A person having origins in any of the original peoples of Hawaii, Guam, Samoa, or other Pacific Islands.
 - **White:** A person having origins in any of the original peoples of Europe, the Middle East, or North Africa
- Reporting of individuals' self-identified countries, regions (e.g., Gullah), or tribes of origin can also be used when known and appropriate.
- Discouraged terms and usages include:
 - People of color
 - Black, Indigenous, and people of color (BIPOC) and Black, Asian, and minority ethnic (BAME) (commonly used in the UK)
 - Terms related to colors, such as brown and yellow

- Americans)
 - The term Caucasian used to indicate the term White
 - Merging of race and ethnicity with a virgule as “race/ethnicity” (Instead, “race and ethnicity” is preferred)
 - Abbreviations of categories for race and ethnicity
 - The term minorities (Instead, use terms that reflect the scientific question such as underserved or underrepresented populations)
 - The term mixed race (instead use multiracial or multiethnic)
- The names of races, ethnicities, and tribes should be capitalized. Examples include: African American, Alaska Native, American Indian, Asian, Black, Cherokee Nation, Hispanic, Kamba, Kikuyu, Latino/a, and White.
- Capitalize the first word and use lowercase for “people” when describing persons who are Indigenous or Aboriginal (e.g., Indigenous people, Indigenous peoples of Canada, Aboriginal people).
- In text and tables, racial and ethnic categories should be listed in either alphabetical order or from largest to smallest group.
- All affected categories of participants should be reported on when possible. If relevant groups are omitted or underrepresented, this should be noted in the Discussion.

Researchers should adjust for objective confounding variables when comparing racial and ethnic groups, and unadjusted findings should be clearly labeled in conjunction with adjusted findings for comparison purposes. If applicable, include racism, discrimination, social class, personal or family wealth, environmental exposures, insurance status, age, diet and nutrition, health beliefs and practices, educational level, language spoken, religion, tribal affiliation, country of birth, parents' country of birth, length of time in the country of residence, and place of residence.

If reporting community-based participatory research (CBPR) results, authors should describe how they engaged with members of populations reported (at outset, throughout the research process), or discuss implications for that population and how findings will be shared and used.

The term sex should be used when reporting biological factors and gender should be used when reporting gender identity or psychosocial or cultural factors.

Sexual designations should be used as adjectives rather than nouns. For example, use “male subjects” instead of “males,” and “female subjects” instead of “females.”

Checklist

1. Explanation of how participant race and ethnicity were assessed in Methods section?
2. Have authors used NIH terminology for race and ethnicity? Are no discouraged terms used?
3. Have authors capitalized the names of races, ethnicities, and tribes?
4. Are racial and ethnic categories listed in either alphabetical or from smallest to largest group?
5. Is there appropriate representation of affected categories of participants based on the condition under study? If not, is this noted in the discussion?
6. Have researchers adjusted for confounding factors when comparing racial and ethnic groups, and clearly labeled unadjusted findings?
7. In the context of CBPR, have authors described how they engaged with members of populations reported or discussed implications for that population and how findings will be shared and used?

Data sharing policy

Authors are required to provide a Data Sharing Statement to be published along with accepted manuscripts. This statement should indicate whether data will or will not be shared.

The manuscript submission system includes specific questions regarding data sharing, including the following options:

- No data were used in this article;
- No data other than what is included in this article or supplementary material will be shared;
- All data used in this research are already included in the article or the supplementary material;
- Data will be shared upon request;
- Data will be shared in a public repository with accession information specified within the article;
- All data used in this research are already publicly available.

If authors elect to share data they will be asked to provide the following information:

- What type of data will be shared,
- Whether additional related documents will be available (e.g. study protocol, statistical analysis plan, informed consent form, codebooks),
- When the data will become available and for how long, by what access criteria data will be shared (including with whom, for what types of analyses, and by what mechanism),
- Whether there will be any costs associated with data sharing.
- For data deposited in repositories, authors will be asked to provide the name or link to the repository used to share data and the accession number to access the shared data.

A list of public repositories that authors can consider for data sharing is available from the **National Library of Medicine**. Authors also have the option to explain why data will not be shared.

Potential Repositories for Data Types

- **Protein sequences:** Uniprot (<http://www.ebi.ac.uk/uniprot>)
- **DNA and RNA sequences:** Genbank (<http://www.ncbi.nlm.nih.gov/genbank/>)
- **DNA:** DataBank of Japan (DDBJ) (<http://www.ddbj.nig.ac.jp/>)
- **EMBL:** Nucleotide Sequence Database (ENA) (<http://www.ebi.ac.uk/ena>)
- **DNA and RNA sequencing data:** NCBI Trace Archive (<http://www.ncbi.nlm.nih.gov/Traces/home/>), NCBI Sequence Read Archive (SRA) (<http://www.ncbi.nlm.nih.gov/sra>)
- **Genetic Polymorphisms:** dbSNP (<http://www.ncbi.nlm.nih.gov/snp>), dbVAR (<http://www.ncbi.nlm.nih.gov/dbvar/>), European Variation Archive (EVA) (<http://www.ebi.ac.uk/eva/>)
- **Linked genotype and phenotype data:** dbGAP (<http://www.ncbi.nlm.nih.gov/gap>), The European Genome-phenome Archive (EGA) (<https://www.ebi.ac.uk/ega/>)
- **Microarray data (must be MIAME compliant):** Gene Expression Omnibus (GEO) (<http://www.ncbi.nlm.nih.gov/geo/>), Array Express (<http://www.ebi.ac.uk/arrayexpress/>)

Related manuscripts/Duplicate publication

Related manuscripts should be sent along with the submitted manuscript. (For example, manuscripts resulting from the same study.)

- Submitted to another journal or *Arthritis & Rheumatology*

Arthritis & Rheumatology receives articles for exclusive publication **only** with the understanding that they:

- have not been published elsewhere (in part or in full, in other words or in the same words, in letter or article form, or otherwise)
- are not at the time of submission under consideration by another journal or other publication
- will not be submitted elsewhere unless rejected by *Arthritis & Rheumatology*.

Articles previously published online, prior to peer review, on preprint servers such as BioRxiv or medRxiv are exempt from this rule and are not considered duplicate publication.

If an author violates this requirement or engages in other similar misconduct, the Editor may, in addition to rejecting the manuscript, impose a moratorium on the acceptance of new manuscripts from the author and, if the misconduct is deemed sufficiently serious, may refer the matter to the author's academic institution or hospital and/or the appropriate state or local disciplinary body.

Clinical trials

Clinical trials must be registered at www.clinicaltrials.gov, www.controlled-trials.com, or another appropriate national body before they will be considered for publication in *Arthritis & Rheumatology*.

This applies to:

- prospective studies of drugs
- biologic agents
- devices
- prospective investigations of prevention strategies
- screening procedures
- diagnostic algorithms
- health promotion approaches
- behavioral interventions
- health care economics.

Exempt are:

- Phase I clinical trials
- pharmacokinetic studies
- studies that do not involve a concurrent control group
- retrospective reviews
- observational studies that do not involve interventions

Additional details are available in an [editorial](#) published in the journal.

Animal studies

Authors of articles reporting research on animals must follow the ARRIVE guidelines

[ARRIVE \(Animal Research: Reporting of In Vivo Experiments\) guidelines and checklist](#).

At the time of submission, a completed ARRIVE checklist must be uploaded.

- number of animals used
- species, strain, sex, and genetic background
- details of housing and husbandry
- experimental, statistical, and analytical methods (including details of methods used to reduce bias, such as randomization and blinding)

Following the ARRIVE guidelines will enable proper critical assessment of the methods and the presented findings and expedite peer review. Submitted articles that include animal studies will not be reviewed without submission of the ARRIVE checklist.

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Upon acceptance (except letters to the editor and replies, which are exempt), the corresponding author will receive an email with a link to the Copyright: Wiley Author Licensing Service.

The Author must promptly complete and electronically sign the appropriate copyright and/or license agreement. **The article will not be published in an issue if this has not been done.**

For authors signing the copyright transfer agreement

If the open access option is not selected the corresponding author will be presented with the copyright transfer agreement (CTA) to sign. The terms and conditions of the CTA can be previewed in the samples associated with the Copyright FAQs below:

CTA Terms and Conditions: http://exchanges.wiley.com/authors/faqs---copyright-_301.html

Open Access

Arthritis & Rheumatology offers authors an open access publication option. You can [read more about APCs](#) and whether you may be eligible for waivers or discounts, through your institution, funder, or a country waiver.

Editorial decisions at *Arthritis & Rheumatology* are independent of the processing of APC payments and waivers.

If the Open Access option is selected the corresponding author will have a choice of the following Creative Commons License Open Access Agreements (OAA):

- Creative Commons Attribution Non-Commercial License OAA
- Creative Commons Attribution Non-Commercial No-Derivs License OAA

To preview the terms and conditions of these open access agreements please visit the [Copyright FAQs](#) hosted on Wiley Author Services and visit

<http://www.wileyopenaccess.com/details/content/12f25db4c87/Copyright--License.html>

If you select the open access option and your research is funded by certain funders [e.g. The Wellcome Trust and members of the Research Councils UK (RCUK) or the Austrian Science Fund (FWF)] you will be given the opportunity to publish your article under a CC-BY license supporting you in complying with your Funder requirements. For more information on this policy and the Journal's compliant self-archiving policy please visit: <http://www.wiley.com/go/funderstatement>.

To preview the terms and conditions of these open access agreements please visit the [Copyright FAQs](#) hosted on Wiley Author Services and visit

<http://www.wileyopenaccess.com/details/content/12f25db4c87/Copyright--License.html>

Author policies

Authorship Criteria

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